City General Hospital — Department of Internal Medicine

SYNTHETIC TEST RECORD — NOT A REAL PATIENT — FOR SOFTWARE QA ONLY

Patient Name:	Priya Sharma	**Patient ID:**	PT-2026-002
Age / Gender:	45 years / Female	**Report Date:**	08-08-2026
Referring Physician:	Dr. Anil Mehta	**Department:**	Endocrinology

CHIEF COMPLAINT

Increased thirst, frequent urination, and fatigue for the past 2 weeks. Patient reports blurred vision and unintentional weight loss of approximately 3 kg.

DIAGNOSIS

1. Type 2 Diabetes Mellitus, uncontrolled
2. Diabetic Ketoacidosis, suspected
3. Stage 2 Hypertension
4. Diabetic Nephropathy, early stage

CURRENT MEDICATIONS

1. Metformin 500 mg — twice daily, oral
2. Glimepiride 2 mg — once daily, oral, morning
3. Ranitidine 150 mg — twice daily, oral, for gastritis
4. Amlodipine 5 mg — once daily, oral, for hypertension

LABORATORY RESULTS

Test	Result	Unit
Fasting Blood Glucose	385	mg/dL
HbA1c	11.4	%
Serum Potassium	6.8	mEq/L
Serum Sodium	128	mEq/L
Serum Creatinine	2.1	mg/dL
Hemoglobin	9.2	g/dL
Serum Cholesterol Total	245	mg/dL

CLINICAL NOTES

Patient reports non-adherence to prescribed medication over the past month due to financial constraints. Vitals on presentation: BP 158/98 mmHg, HR 102 bpm, RR 20/min, afebrile. Mild dehydration noted on examination. Referred for urgent endocrinology consultation and inpatient monitoring given elevated potassium and suspected DKA.

— End of report — Dr. Anil Mehta, MD (Internal Medicine), City General Hospital